

The new REF individual circumstances setup: the good and the bad

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I am quoted in [today's Research Fortnight](#) on the new REF staff/individual circumstances under the heading 'REF staff circumstances rules criticised'. The quote used only gives a selection of the views that I supplied and omits the bits where I say why the new situation might be better (that's fine – it's journalism and I was not quoted inaccurately). Here are my full remarks in context:

What are your personal experiences of REF staff circumstances?

I have applied for individual circumstances having had a stroke, pneumonia and sepsis, neurological hearing loss, and near bowel-perforation – all as a result of my vasculitis – during this REF cycle. Clearly, these have impacted on my research, although I note that I have a comprehensive REF portfolio already.

What are your concerns?

1. The new REF rules can seem odd. The reduction is applied at the unit level, not to the individual researcher – unless it's a reduction to zero outputs. So I can be submitted with the maximum number of outputs but the unit receives a reduction. (See below on benefits.)

2. It's incredibly invasive. Despite rigorous data protection policies, it's pretty traumatic having to reconstruct a history of severe medical problems, rereading old notes etc. and then to communicate these to your institution. I suspect many people are also fearful of disclosing such information for fear of pushback.
3. It's very difficult to document! If you haven't kept letters from hospitals, it is very hard to put the narrative together. Also, different letters show speculative diagnoses at different points. That is, earlier letters may appear to contradict more recent changes of medical opinion (which is more an art than a science). This can cause anxiety in documenting what you have. (For instance, I have a letter saying I have Crohn's disease, only for this to be questioned in a later clinic and consultation.)

Are there any benefits to the new system?

The *idea* behind the new system is good. While I said earlier that it seems odd that I should apply for circumstances even while it is not my output count that is reduced, the new system *should* encourage institutions to work out how workloads can be distributed *within a unit* to allow staff with disabilities, illnesses, and impairments to conduct research. You could argue that the reason I was able to be so productive even while seriously ill is that my institution and colleagues have worked to support me through such challenges – perhaps meaning that others have fewer outputs.

I should also add that institutions cannot pressure staff (and did NOT pressure me) to apply; so it is good that this is a voluntary process, although this may not be the case for those who are fighting to stay on a research contract although not having any outputs. Indeed, except in this latter case, it is beneficial for institutions to support their staff to apply for this.